جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نورا محمد عبد الستار محمد
المحافظة : القاهره **الحي / المدينة :** خمسة عشر مايو **العنوان :** =
رقم البطاقة : 28211020102329 **رقم التليفون :** 01226028010 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 02-11-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نورا محمد عبد الستار محمد
National ID: 28211020102329

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS

مزيد من الأبحاث والفحوصات



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

03/31/2026

MDT FORM

Name:	Noura Mohamoud Abd Elsatar Mohamoud	MRN:	5416
Age:	43 Y	ID:	28211020102329
Menopausal Sta		Phone:	01226028010
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC

ER: 8/8 **PR:** 3/8 **HER2:** 0 **KI67:**

Sonomammography:

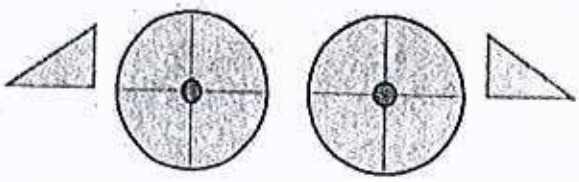
Mets Workup:

History:
A CASE OF RT BREAST CANCER SINCE 2022
RT MRM ..ADJ CTH PORTH THEN HT AI ZOLADEX TILL 8/2025
PT DEVELOPED BONE LIVER LN AND CHOROID METS
STARTED IBRANCE FASLODEX ZOLADEX FOR 3 MONTH
EVALUATION BY PET CT SHOWED DP SO PT SHIFTED TO TAXOTERE *3 CYCLES
PETCT 3/2026 SHOWED MIXED RESPONSE

Recommendations:
DR TAMER ELNAHAS RECOMMEND GEMZAR CARBOPLATIN ZOMETA

MDT Outcome & Plan
for GEMZAR CARBOPLATIN ZOMETA

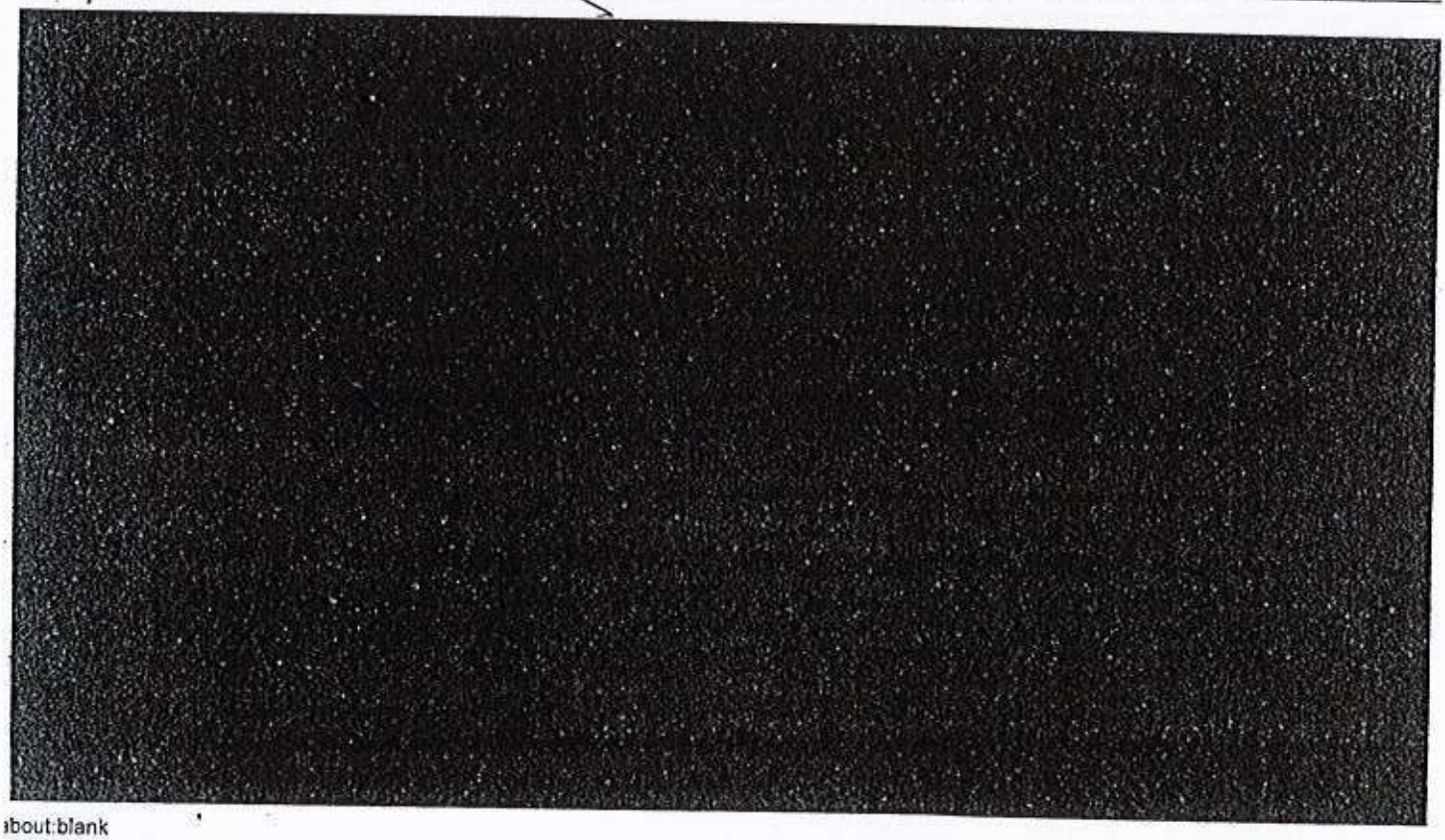
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat					



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: نورا محمد عبدالستار محمد	
MR. : 5416	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

mbc to left choroid of eye, liver, lung and ln progressed on 2nd line hormonal ttt by pet/ct 11/2025 then received 4 cycles taxotare last today 2/2/2026 for new assessment but ptn cannot afford so for new cts compared to last one in 11/2025 and for mri head and neck to comment on left choroid which is had regression in last pet/ctb in 11/2025

A CASE OF MBC TO BONE LUNG LIVER AND LN HR POSITIVE HER2 NEGATIVE IDC GRADE 2 RECEIVED IN METASTATIC PHASE IBRANCE FASLODEX ZOLADEX SINCE 6/2025 TILL 3/2026 PET CT SHOWING MIXED RESPONSE FOR MDT PT IN SEVER PAIN FOR PAIN CLINIC CONSULTATION

43 y wt 45 ht 155 sa 1.38 prem mbc to liver lung l nodes choroid and recently brain and bone new hfl 11/2025 met carcinoma er8/8 pr3/8 her2-ve ki 50 history ;2021 rt mrm t3 n1 mo rec adj ac x3 txt x3 port ai zoladex till6/2025 mets to liver lung ln st fasoldex ibrance till11/2025 dp rec taxotere x3 cycles pet ct 11/3/2026 dp of hfl newly dev hfl newly developped osseous lesions med ln mdt gemzar carbopl zometa pt rec pall rth whole brain irradiation and to c1-c7,d1d2 ,d6 to d12 and l2to 5 left hip from 20/4/2026 to 3/5/2026 for c1 gemzar 1380 carboplatin auc 2.5 and assess tolerability next week and liver support

cc vomiting constipation for c2 d1 gemzar 1380 carboplatin auc 5 and zometa and liver support and blood transfusion and repeat cbc next week

today for c2 d1 gemzar carboplatin auc 5, labs within accepted range, for liver support d8 pharmacy with CBC

tlc 0.4 hb 7.6 plt 106 for blood transfusion postpone 1 week and consider dose reduction

bec of grade 2 thrombocytopenia for 10% dr and fractionate carbopl over 2 weeks for c2 d1 gemzar 1240 carboplatin auc 2

for c2 d8 gemzar 1150 carbopl auc 2 with 2 proph neupogen tlc1.9/1.4 hb8.5 plt 130 bec pt was delayed mutiple times with further 10 % dr

tlc 1.9/0.9 to postpone 1 week consider omission of carboplatin and give gemzar only bec of multiple delay bec of neutropenia inspite of dr and proph neupogen

cbc ok for c3 d1 gemzar 1150 carboplauc 2 neoadj and 2 proph neupogen

omit carboplatin for the time being and proceed with c3d1 gemzar only (CBC done outside TLC 2.5 ANC 1.3) with proph neupogen, consider readding carboplatin if cbc improves

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

for c3 d8 gemzar 1150 carbopl auc 1.5 and blood transfusion for new cts after this cycle

مستشفى روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 23 / 11 / 2025 G

Name	نورا محمد عبد الستار	MRN	2741/2025
		ID	٢٨٠١١٠٢٠١٠٢٢٠٩
Age	43 yrs	Phone	٠١٥٢٦٠٢٨١٠٠
Menopausal status	pre	Marital status	M
NRS SCORE		Family history	+ve
BMI		REASON OF MDT	Dp
CO morbidity		PS I	

Clinical Data

BX: IDC, GII

ER:	PR:	HER 2:	KI67:
718	618	-ve	30%

R L

Sonomammography:

Mets Workup:

(New) PET Scan (18/11/2025) (overall Progressive)
- Newly developed BM + LNs lesions
- Breast & HFL (5x3,2) Captured (2cm)

TNM (M) Liver LNs Bone Lung

Recommendations:

2021 Rt MRM → $pT_3N_1M_0$ Received Adjuvant [AC X 3, TXT X 3] +RT

Kept on Adjuvant AI + Zoladex till (6-2025)

PET Scan (6-2025) :- Multiple Nodal, lung, liver mets

& start Zoladex + Fasledex + Ibrance (From 8-2025 till Now)

and Received palliative RTH to chest.

MDT OUTCOME & PLAN

1) b ReBiopsy L HFL
2) Hostet Taxotere q 3 weeks L/B

Herceptin Adj. 6/11/19

for taxotere X3 cycles + Biopsy

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
				Re MDT	If HER 2 +ve

Scanned with CamScanner

National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

Pathology Report

Name: نورا محمد عبدالمنعم Age: 37 years.

Clinical data: Two Right breast masses (10 O'clock and 12 O'clock)

Specimen type: Tru cut biopsy.

Date (Received): 9/5/202. Date(reported): 21/6/2021

Lab. Number: 1192/21 Hospital Number: 21/صحة المرأة

Gross Examination:

Two specimens were received:

1- 3 greyish white and hemorrhagic tissue cores up to 1cm. Totally submitted.
2- 4 fragmented greyish brownish tissue cores up to 0.4cm. Totally submitted.

Microscopic examination:

Both specimens revealed an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells and focal insitu ductal components. There is moderate cellular and nuclear atypia. Stromal desmoplasia is moderate.

Diagnosis:

Two Right breast masses, Tru cut biopsy.

Both specimens showed invasive duct carcinoma grade II, with focal insitu ductal components.

For markers study (ER, PR, Her-2 and Ki67).

Prof. Dr. Lubna Omar

Dr. Abdulrahman Ibrahim

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

- **Left eye:**
  - Mild thickening of the nasal aspect of the left choroid demonstrating mild FDG uptake, SUVmax 2.7 and measures 0.4 cm in thickness ***(compared to SUVmax 3.0 and 0.4 cm in thickness)***
- **Right eye,** no sizable FDG avid focal lesions.
- **Brain,** no FDG avid lesions.
- Diffuse FDG uptake by the gluteal muscles bilaterally being more focal on the left side with no definite underlying CT changes, SUVmax 4.9, ***for follow-up.***
- Diffuse FDG uptake along the stomach, likely inflammatory, for clinical correlation.



II. Diagnostic CT Findings:

- No pleural, pericardial or peritoneal collections.
- The spleen, pancreas, adrenals, kidneys, pelvic organs, aorta and IVC appear unremarkable.

OPINION:

- **Metabolically active metastatic lymph nodal, pulmonary, hepatic and bone marrow deposits.**
- **Low grade metabolic activity by left choroidal thickening.**

Based on the newly developed bone marrow and lymph nodal lesions, there is overall disease progression.

Radiologists
Dr. Mohamed Shahin, MD
Dr. Mohamed Hoseny, MD

Nuclear Medicine Physicians
Dr. Mohammed Omar, MD
Dr. Abdelhamid Abdou, MSc

PET Scan
[[DOCTYPE:PET_SCAN]]

- **Liver:**
  - Increased FDG uptake by **segment VI subcapsular hypodense focal lesion**. SUVmax 9.7 and measures 5 x 3.2 cm ***(compared to previously reported SUVmax 9.7 and 2 cm)***
  - Patent portal vein.
  - No dilated intrahepatic biliary radicles.



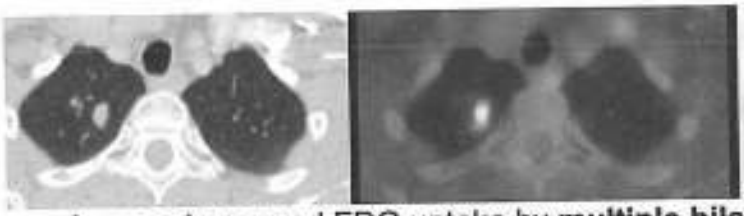
- **Bone marrow,** FDG-avid multiple lesions involving skull base, scapulae, bilateral ribs, multiple vertebrae, sacrum, pelvic bones and left femur ***(newly developed),*** SUVmax 10.1 as measured over a left iliac bone lesion. SUVmax 11.1 as measured over D7 lesion. SUVmax 9.2 as measured over C2 lesion. SUVmax 8.9 as measured over left 6th rib posterior aspect lesion. Few lesions are associated with cortical erosions and soft tissue component.



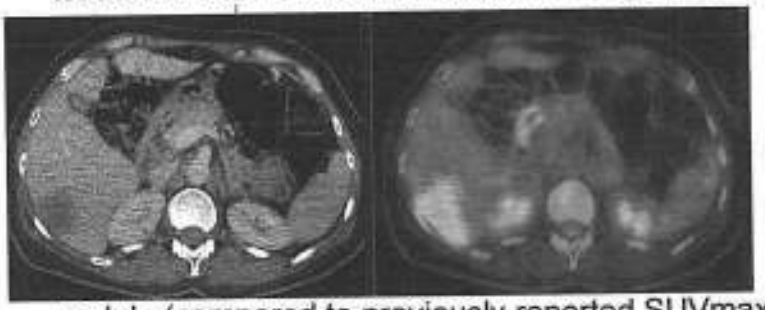
PET Scan
[[DOCTYPE:PET_SCAN]]



- **Cervical LNs,** including bilateral level II and supraclavicular LNs as well as left level III LNs. The left supraclavicular LNs appear partially amalgamated with SUVmax 14.5 and globally measuring 2.8 cm (compared to previously reported SUVmax 7 and 1 cm).
- **Abdominal LNs,** including left para-aortic and portahepatis LNs, the latter has SUVmax 5.9 and measures 1.3 cm ***(newly developed)***



- **Lungs,** increased FDG uptake by **multiple bilateral pulmonary nodules.** SUVmax 8.2 as measured over 1.1 cm right upper lobe



nodule (compared to previously reported SUVmax 10.1 and 1.1 cm), SUVmax 6 as measured over 1.1 cm left upper lobe nodule (compared to previously reported SUVmax 6.3 and 1.1 cm)

PET Scan
[[DOCTYPE:PET_SCAN]]

Name: Nora Mohamed Abd Al Sattar Mohamed *ID: 4530530*
Date: 18/11/2025

^{18}F-FDG PET/CT examination

Clinical Indication:

- *Patient with history of:*
  - *Surgically excised breast cancer in 2021.*
  - *Left choroidal thickening, received interval radiotherapy.*
  - *Metastatic lymph nodal, pulmonary and hepatic deposits, received targeted therapy, referred for response evaluation.*

Administered activity: 240.5 MBq. ^{18}**F-FDG** **B.G.L.** : 93 mg/dL

Examination protocol:

- **A Whole body PET/CT study was obtained from skull vertex to feet using a low dose non contrast CT for attenuation correction**
- **Diagnostic multislice CT examination after IV contrast administration of the same region.**

Reference was made to previous FDG PET/CT study report 21 June 2025. No DICOM images available for comparison.

I. ***PET/CT Findings:***

- **Right mastectomy operative bed,** no FDG avid lesions.
- **Left breast,** no FDG avid lesions.



- **Lymph nodes, increased FDG uptake corresponding to:**
  - **Mediastinal LNs,** including paratracheal, prevascular, aor topulmonary, tracheobronchial, subcarinal, retrosternal and bilateral hilar lymph nodes. SUVmax 9.7 as measured over 3.3 cm right paratracheal LN (compared to previously reported SUVmax 10.7 and 3.3 cm.

PET Scan
[[DOCTYPE:PET_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Nora Mohamed Abd Al Sattar Mohamed	Date : 09/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 2741/2021	Pat. ID: 12901

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- A small fat containing umbilical hernia.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

Status follow up revealed:

- **No appreciable changes regarding the mediastinal lymphadenopathy.**
- **Progression of the bilateral pulmonary metastatic nodules.**
- **Mild progression of the right hepatic lobe focal lesion? Metastatic.**

Signature Prof. Ali Remah
11/11/2025 05:41 م
Signature Prof. Ali Remah
18/11/2025 04:15 م

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Nora Mohamed Abd Al Sattar Mohamed	Date : 09/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 2741/2021	Pat. ID: 12901

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

****Status follow up after reviewing the previous PET/CT study, the current study revealed 21/06/2025**

I Chest:

- Evidence of right mastectomy.
- Multiple enlarged lymph nodes are seen at the prevascular, retrocaval, pretracheal, paratracheal, carinal and subcarinal regions, largest measurinmg 2.5 x 1.5 cm in axial dimensions (no appreciable changes).
- Multiple bilateral pulmonary nodules are seen scattered at both lung fields, largest measuring 13 mm in diameter (increased since prior study).
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. Right hepatic lobe focal lesion (segment VI) measuring 3.6 x 2.7 cm in axial dimensions (progressive since the prior study).
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nora Mohamed Abd Al Sattar Mohamed	Date : 08/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 42 Year(s)
Medical ID: 2741/2021	Pat. ID: 12901

LEFT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

08/01/2025 4:06 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

الرقم الطبي:	5416
اسم المريض:	نورا محمد عبدالستار محمد
تاريخ الميلاد:	02/11/1982
الرقم الطبي القديم:	null

Blood Transfusion Monitoring Form

Date of delivery: 5.../9.../26 Time of delivery: 2.:18ph Transfusion start time: 2.:25 End Time: 4.:15ph

Blood Group & RH Type: ...A... Type of blood unit: .positive. Blood unit No: .....

Blood unit double checking:	☐ Assigned nurse	Name & ID: Afaf Khairy 3025 Signature: Afaf Khairy 3025	☑ Charge Nurse	Name & ID: Azza Said 3012 Signature: Azza Said 3012
Patient Identification double checking (bedside):	☑ Assigned nurse	Name & ID: Afaf Khairy 3025 Signature: Afaf Khairy 3025	☑ Charge Nurse	Name & ID: Azza Said 3012 Signature: Azza Said 3012

Nursing Observation: ☑ No reaction ☐ Reaction (Rash, Fever, Shivering, Facial edema, Laryngeal Spasm, ...)

If reaction, Notified to: .....Time: ... : ... Documented in Blood Transfusion Reaction Form ☐ Yes ☑ No

	Time	Temp.	Pulse	SPO2	RR	BP	Pain
Baseline vital signs	2:25	36,1	80	99	19	110/70	–
After 5 minutes	2:30	36,1	85	98	19	110/70	–
After 10 minutes	2:35	36,5	79	99	19	110/70	–
After 15 minutes	2:45	37,-	77	98	19	110/70	–
After 30 minutes	3:ph	37,-	71	99	18	100/80	–
After 45 minutes	3:10	37,–	73	96	19	100/70	–
After 60 minutes	3:25	37,–	75	98	18	110/70	–
After 90 minutes	4:ph	37,–	78	99	17	110/70	–
After completion of transfusion	4:15ph	37,–	75	99	16	120/70	–
After 5 minutes from completion of transfusion	4:20ph	37,-	78	99	20	120/70	–
After 1 hour from completion of transfusion	5:15ph	37,-	75	95	18	110/70	–
After 2 hours from completion of transfusion	6:15ph	37,1	70	98	20	110/70	–

Nurse Name & ID: Afaf khairy 3025 Signature: Afaf Date: 5../9./2026 Time: 8..:30ph

* Use separate form for each unit transfused.

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

DOCTOR'S ORDERS

الرقم الطبي:	5416
اسم المريض:	نورا محمد عبدالستار محمد
تاريخ الميلاد:	02/11/1982
الرقم الطبي القديم:	null

Diagnosis: B.C Unit: Daycare Diet Type: Normal

Date	Time Of Order	Doctor's Order	Physician Name & ID	Signature	Nurse Name & ID	Signature	Time of Order Execution
5/9	1pm						
					Afaf Khairy	3:25	2:25pm

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.
* Do NOT use abbreviations.
*This form is NOT intended for use for medications orders.